5.12 Autonomic Dysreflexia*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have a pounding headache." **Objective** • Injury at T6 or above • BP 200/110 mmHg, HR 50/min • Flushing and sweating above the injury level	**ACTUAL** Acute pain related to sympathetic hyperactivity triggered by a noxious stimulus below the level of injury as evidenced by a pounding headache with severe hypertension and bradycardia.	• BP back to baseline once the trigger is removed.	• Sit him upright and lower his legs. • Loosen any tight clothing. • Check the bladder (kinked catheter, distension) and the bowel (impaction). • Measure BP every 2–5 minutes. • Give the prescribed antihypertensive if BP stays high.	• Achieved when BP normalizes and the headache settles.
Subjective • "What caused this?" **Objective** • First episode • Cannot name the triggers	**ACTUAL** Inadequate health knowledge related to prevention of autonomic dysreflexia as evidenced by questioning.	• Patient and family can name the triggers and the immediate actions.	• Teach the common triggers: full bladder, constipation, tight clothing, pressure injury. • Teach the emergency steps: sit up, loosen clothing, check the bladder. • Give him an emergency information card.	• Outcome reached once explained correctly.
Subjective *No specific complaint* **Objective** • Spinal cord injury at T4	**RISK** Risk for autonomic dysreflexia as evidenced by risk factors: spinal cord injury at T6 or above.	• Episodes prevented or treated at once.	• Keep to the bladder and bowel programs. • Check skin and clothing for pressure. • Teach the family the emergency steps.	• Considered met if no episodes occur.

* **References for this care plan:** [32], [35], [37], [128]. Full entries are listed in the References section of the CPMN book.